

Community Participation: Individualized Day Habilitation Services provide community-based habilitative services for individuals who require support to develop skills that lead to independent community navigation. Participants must have a desired outcome to access to the community in a meaningful way via true community integration. Paid staffing supports are required to fade as natural community supports are developed. Each individual receiving Community Participation services works toward acquiring the skills to become an active member of their community. Community Participation services are highly individualized, thus assumes a staff to client ratio of 1:1 or 1:2. Although Community Participation activities may be provided in a location that would be considered to be recreational in nature, the purpose of the environment is to provide the opportunity to acquire the identified skill.

Employment First Act 2012 declares that "People with disabilities have a right to the opportunity for competitive employment. In order to achieve meaningful and competitive employment for persons with disabilities, employment opportunities in fully integrated work settings shall be the first and priority option explored in the service planning for working age persons with disabilities."

- It must be documented that the right to the opportunity to explore competitive employment was fully explained during the service planning process.
- It must be documented that the various available employment services were fully explained during the service planning process.
- The individual's informed decision about accessing employment services must be documented
- All service recipients expressing an interest in employment must be referred to the Division of Vocational Rehabilitation to be assessed for eligibility.

Direct Support Staff providing Community Participation services may perform the following activities:

Explore/Discover Individual Interests - Community Based assessments (non-employment related), informational interviews with the participant, family and others who know them well, administer interest inventory tools

Identify Community Resources – Based on Individual interests and skill sets: make cold calls, network with family and other support resources, reach out to existing community partners, attend county/community events, community mapping

Maintenance of appropriate community and interpersonal behaviors - develop and implement fading plan, maintain appropriate community behaviors, follow-up as needed with the individual to determine needs and offer encouragement and/or advice, and

Other training needs identified by assessment and by the individualized support plan -This may include some of the following: basic money management, coordinate natural supports, the use of public transportation, on-going assessment and support in development of future community participation goals

Direct Support Staff providing Community Participation services have:

- A. Successfully completed a Community Integration Curriculum as approved by DDDS. Completion must occur within six months of date of hire; Persons who have not completed an approved Community Integration Curriculum and who are currently providing services as of the effective date of this standard shall have six months to come into compliance;

- B. Received increased support, supervision and guidance during their first 6 months of employment by a qualified mentor. A qualified mentor is someone who has worked for a minimum of one year providing community integration supports and/or supported employment supports for people with disabilities
- C. Graduated from high school or acquired a GED; Persons without a high school diploma or a GED and currently providing the services as of the effective date of this rule shall have three years to obtain the minimum educational requirements
- D. A background check consistent with Provider Standards issued by the DDDS Office of Service Enhancement and Integrity
- E. A valid Driver's License and valid vehicle insurance if transporting clients
- F. Unique skill sets similar to those staff who do effective job placement and support. The following competencies may be important for a staff who is providing Community Participation services:
 - ✓ Person-Centered Planning
 - ✓ Community mapping
 - ✓ Networking
 - ✓ Negotiating skills
 - ✓ Facilitation of integration in community settings
 - ✓ Maximizing use of natural supports
 - ✓ Individual support strategies: accommodations, assistive technology, systematic instruction, etc.
 - ✓ Supporting individuals with complex needs in community settings

Community Participation Services shall include activities that directly relate to the service recipient's desired outcomes.

- Career Exploration: assessing of interests through volunteer experiences, internships, etc.
- Community integration experiences to support fuller participation in community life
- Skills development and training
- Development of Activities of Daily Living and Independent Living skills
- Socialization experiences and support to enhance interpersonal skills
- Pursuit of personal interests and hobbies.

Activities should be motivated by the following core principles:

- Maximum time in the community
 - Deliberate and purposeful activities
 - Driven by individual outcomes and preferences
 - Supplement to employment, but not a replacement
 - Use generic community resources
 - Adult appropriate activities
 - Focus on relationship development with community members
 - Maximize community/natural supports
 - Staff model interactions with individuals that foster an image of competence and promote acceptance
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- Community Participation services promote full integration and inclusion of service recipients into mainstream society through positive community contributions and relationship development
- Community Participation services are not activities to “fill” a the service recipient’s day; it is a vehicle for service recipients to explore and engage in meaningful and purposeful activities, based on individual preferences and needs as identified in their person-centered services and supports plan
- The individual’s person centered services and supports plan must include Community Participation as a desired service and their outcomes, goals, services and supports must be consistent with the services defined as Community Participation Services. The individual’s person-centered services and supports plan **must** include specific outcomes related to community integration
- The Community Participation outcomes, goals and the person-centered services and supports plan must be reviewed: no less than annually, more frequently as necessary or as requested by the individual.
- Community Participation Services are expected to be provided only for as long as necessary. A goal to decrease professional supports and increase natural supports **must** be a standard part of every individual’s plan. Specific fading strategies **must** be outlined in the person-centered services and supports plan.
- Community Participation services may also be used to provide supported retirement activities. As some people reach retirement age, they may no longer desire to work and may need supports to assist them to engage in meaningful retirement activities in their communities. This might involve altering schedules to allow for more rest time throughout the day, support to participate in hobbies, clubs and/or other senior related activities in their communities that provide a specific beneficial outcome for the recipient.
- Community Participation services take place in a non-residential setting that is separate from the participant’s private residence or other residential living arrangement.
- Individuals may gather at the beginning and the end of the service day at a “hub” before embarking on their activities of the day or returning home, but they may not spend more than one hour in total at the hub per day. Other than the brief period at the beginning and end of the day, Community Participation services cannot be delivered in a provider owned or managed setting.
- Agency staff may provide personal care/assistance as a component part of Community Participation services in order to meet the needs of a participant, however personal care/assistance may not be the **only** service provided.
- Community Participation is not intended to be used as supports for paid employment activities
- Individuals receiving Community Participation services may also receive Supported Employment, Pre-vocational, Day habilitation services and/or any other Day Service. A participant’s person-centered services and supports plan may include two or more types of Day Services; however, different types of day services may not be billed for the same part of the day. Additionally activities billed as Community Participation services cannot also include activities identified as billed as Vocational Rehabilitation services

- Community Participation services may be provided outside of the “typical” daytime hours; however, for service recipients who also receive Residential Habilitation, Supported Living or any other Residential “like” Support Service, documentation must be included as part of their person-centered services and supports plan detailing why those activities cannot be provided as part of the residential service and support activities
- Transportation includes staff time to transport individuals to and/or from their community location. If transportation support is to be provided, it must be described in the individual’s person-centered services and support plan and an outcome to assist the individual to become more independent and access a more integrated form of transportation must also be included in the plan
- Although it is the expectation that services billed as Community Participation Services will include the participant, this expectation does not automatically exclude services provided by staff “on-behalf-of” the participant from being a legitimate, billable service. However, prior to providing and billing for services “on-behalf-of” the participant, the agency must first consider the following:
 - 1) Why should this service be provided as “on-behalf-of” vs. directly engaging the participant?
 - 2) Could the participant benefit by increasing their knowledge/abilities if they were directly engaged in this activity?

It is recommended that agencies clearly document when billing for services provided as “on behalf of” the participant, and include a brief justification for why the services were provided as “on behalf of” vs engaging the participant

Authorization of Service & Billing:

Community Participation is a Medicaid Waiver funded service. The current reimbursement methodology was designed with the expectation that this service be highly individualized; thus permitting either a staff to consumer ratio of 1:1 or 1:2. The maximum number of direct support hours a consumer can receive of Community Participation services within a 5.5 hour program day is determined by the individual’s ICAP scores.

An ***automatic exception*** will be granted ***ONLY*** for the ***initial*** authorization of Community Participation services. This initial exception allows for preliminary discovery activities, assessment activities and coordination of activities and services. The initial exception includes a standard increase of 500 units which will be added to the number of units as determined by the consumers’ ICAP assessment. Any other exception request will be considered if supported by documentation via the Community Services Exception Procedure.

Service authorizations are made in 15 minute billable units. The prior authorization created for each individual indicates the maximum amount of units that can be billed for a specified period of time. It is not a guarantee of payment for the entire amount authorized. Claims may only be submitted for hours of direct support actually provided.

It is critical that all Service Providers maintain Medicaid compliance and ensure services are delivered as authorized by the Division. It is expected that service recipients’ service needs may change. As changes occur, DDDS expects the Service Provider to communicate those changes to DDDS before the service is delivered, but will accept changes communicated within 7 calendar days of when the change occurred.

In the event that a provider provides fewer hours of direct support on any given day than what would be expected based on the authorization, the provider must bill for only those hours of direct support actually provided and supported by documentation.

Billing in excess of the number of direct support hours actually provided or billing contrary to the established reimbursement methodology approved in the DDDS waiver would be considered an overpayment by Delaware Medicaid or CMS. If they were to audit such claims, it would likely result in a recoupment unless the provider had documentation to support the number of direct support hours billed.

If CMS were to determine that the overbilling was purposeful, the Office of Inspector General of the U.S. DHHS has the authority to sanction the provider, which would result in their exclusion from participation in the Medicaid and Medicare programs for a period of 5 years (see regulatory citation below.)
§1003.102 Basis for civil money penalties and assessments.

- (a) The OIG may impose a penalty and assessment against any person whom it determines in accordance with this part has knowingly presented, or caused to be presented, a claim which is for.....
 - (1) An item or service that the person knew, or should have known, was not provided as claimed, including a claim that is part of a pattern of practice of claims based on codes that the persons knows or should know will result in greater payment to the person than the code applicable to the item or service actually provided.

Documentation: Records must be maintained in one or more documents to document the provision of service to an individual, consistent with the individual's service and support plan. At minimum, service documentation must be included in the DDDS approved Electronic Case Record. At least one billable note must be maintained for each service delivered, each service day. Documentation may be in the form of paper or electronic software programs and must be kept in a manner as to fully disclose the nature and extent of services delivered which include, at a minimum:

- Type of Service
- Date of Service
- Location/Place of Service
- Name of Individual receiving service
- Progress the individual made towards the outcomes identified in the individual's service and support plan
- Medicaid ID number of the individual receiving service
- Name of Provider
- Signature (may be electronic) or initials of the person delivering the service (if signature and corresponding initials are on file with the provider)